

BRUTAFLAM (Etoricoxib Tablets)			
PRODUCT NAME BRUTAFLAM-60 (Etoricoxib Tablets 60mg) BRUTAFLAM-90 (Etoricoxib Tablets 90mg) BRUTAFLAM-120 (Etoricoxib Tablets 120mg)			
NAME AND STRENGTH OF ACTIVE SUBSTANCE(S) Each film coated tablet contains 60, 90 or 120 mg of Etoricoxib List of Excipients Core : Calcium Hydrogen Phosphate Anhydrous, Croscarmellose Sodium, Magnesium Stearate, Microcrystalline Cellulose, Hypromellose and Purified Water. Tablet Coating: Hypromellose, Lactose Monohydrate, Titanium Dioxide, Triacetin. The 60 and 120 mg Tablets also contains Indigo Carmine Aluminium Lake and Iron Oxide Yellow.			
PRODUCT DESCRIPTION Film-coated tablets. BRUTAFLAM-60: Dark-green colored, round shaped, biconvex film coated tablet plain on both sides. BRUTAFLAM-90: White, round shaped, biconvex, bevelled edge, film coated tablet plain on both sides. BRUTAFLAM-120: Pale-green colored, round shaped, biconvex film coated tablet plain on both sides.			
PHARMACOLOGICAL PROPERTIES Pharmacodynamics Pharmacotherapeutic group: Anti-inflammatory and antirheumatic products, non-steroids, coxibs, ATC code: M01AH05 Etoricoxib is a nonsteroidal anti-inflammatory drug (NSAID) that exhibits anti-inflammatory, analgesic, and antipyretic activities in animal models. Etoricoxib is a potent, orally active, highly selective cyclooxygenase-2 (COX-2) inhibitor within and above the clinical dose range. Two isoforms of cyclooxygenase have been identified: cyclooxygenase-1 (COX-1) and cyclooxygenase-2 (COX-2). COX-1 is responsible for prostaglandin-mediated normal physiologic functions such as gastric cytoprotection and platelet aggregation. Inhibition of COX-1 by nonselective NSAIDs has been associated with gastric damage and platelet inhibition. COX-2 has been shown to be primarily responsible for the synthesis of prostanoid mediators of pain, inflammation, and fever. Selective inhibition of COX-2 by etoricoxib decreases these clinical signs and symptoms with decreased GI toxicity and without effects on platelet function. Across clinical pharmacology studies, Etoricoxib produced dose-dependent inhibition of COX-2 without inhibition of COX-1 at doses up to 150 mg daily. The influence on gastroprotective COX-1 activity was also assessed in a clinical study where prostaglandin synthesis was measured in gastric biopsy samples from subjects administered either Etoricoxib 120 mg daily, naproxen 500 mg twice daily, or placebo. Etoricoxib did not inhibit gastric prostaglandin synthesis as compared to placebo. In contrast, naproxen inhibited gastric prostaglandin synthesis by approximately 80% compared with placebo. These data further support the COX-2 selectivity of Etoricoxib. Platelet Function Multiple doses of Etoricoxib up to 150 mg administered daily up to nine days had no effect on bleeding time relative to placebo. Similarly, bleeding time was not altered in a single dose study with Etoricoxib 250 or 500 mg. There was no inhibition of ex vivo arachidonic acid- or collagen-induced platelet aggregation at steady state with doses of Etoricoxib up to 150 mg. These findings are consistent with the COX-2 selectivity of etoricoxib. Pharmacokinetics Absorption: In studies specifically designated to measure the onset of action of etoricoxib, the onset of action occurred as early as 24 minutes after dosing. Orally administered etoricoxib is well absorbed. The mean oral bioavailability is approximately 100%. Following 120 mg once-daily dosing to steady state, the peak plasma concentration (geometric mean C_{max} = 3.6 mcg/mL) was observed at approximately 1 hour (T_{max}) after administration to fasted adults. The geometric mean AUC_{0-24h} was 37.8 mcghr/mL. The pharmacokinetics of etoricoxib are linear across the clinical dose range. A standard meal had no clinically meaningful effect on the extent or rate of absorption of a dose of etoricoxib 120 mg. In clinical trials, etoricoxib was administered without regard to food. The pharmacokinetics of etoricoxib in 12 healthy subjects were similar (comparable AUC, C_{max} within approximately 20%) when administered alone, with a magnesium/aluminium hydroxide antacid, or a calcium carbonate antacid (approximately 50 mEq acid-neutralizing capacity). Distribution Etoricoxib is approximately 92% bound to human plasma protein over the range of concentrations of 0.05 to 5 mcg/mL. The volume of distribution at steady state (V_{ss}) is approximately 120 L in humans. Etoricoxib crosses the placenta in rats and rabbits, and the blood-brain barrier in rats. Metabolism: Etoricoxib is extensively metabolized with <1% of a dose recovered in urine as the parent drug. The major route of metabolism to form the 6'-hydroxymethyl derivative is catalyzed by cytochrome P450 (CYP) enzymes. Five metabolites have been identified in man. The principal metabolite is the 6'-carboxylic acid derivative of etoricoxib formed by further oxidation of the 6'-hydroxymethyl derivative. These principal metabolites either demonstrate no measurable activity or are only weakly active as COX-2 inhibitors. None of these metabolites inhibit COX-1. Elimination Following administration of a single 25 mg radiolabeled intravenous dose of etoricoxib to healthy subjects, 70% of radioactivity was recovered in urine and 20% in feces, mostly as metabolites. Less than 2% was recovered as unchanged drug. Elimination of etoricoxib occurs almost exclusively through metabolism followed by renal excretion. Steady state concentrations of etoricoxib are reached within seven days of once- daily administration of 120 mg, with an accumulation ratio of approximately 2, corresponding to an accumulation half-life of approximately 22 hours. The plasma clearance is estimated to be approximately 50 mL/min.			
INDICATION Etoricoxib Tablets is indicated for:			
• Acute and chronic treatment of the signs and symptoms of osteoarthritis (OA) and rheumatoid arthritis (RA) • Treatment of ankylosing spondylitis (AS) • Treatment of acute gouty arthritis • Treatment of acute pain, including that related to primary dysmenorrhoea and minor dental procedures. The decision to prescribe a selective COX-2 inhibitor should be based on an assessment of the individual patient's overall risks (see PRECAUTIONS). DOSAGE AND ADMINISTRATION Etoricoxib is administered orally. Etoricoxib may be taken with or without food. Etoricoxib should be administered for the shortest duration possible and the lowest effective daily dose should be used. Osteoarthritis (OA) The recommended dose is 30 mg or 60 mg once daily. Rheumatoid Arthritis (RA) The recommended dose is 60 mg once daily. In some patients with insufficient relief from symptoms, an increased dose of 90 mg once daily may increase efficacy. Once the patient is clinically stabilised, down-titration to a 60 mg once daily dose may be appropriate. In the absence of an increase in therapeutic benefit, other therapeutic options should be considered. Ankylosing Spondylitis The recommended dose is 60 mg once daily. In some patients with insufficient relief from symptoms, an increased dose of 90 mg once daily may increase efficacy. Once the patient is clinically stabilised, down-titration to a 60 mg once daily dose may be appropriate. In the absence of an increase in therapeutic benefit, other therapeutic options should be considered. Acute Pain In the following acute painful conditions, Etoricoxib should be used only for the acute symptomatic period, limited to a maximum of 8 days treatment: Acute Gouty Arthritis The recommended dose is 120 mg once daily. Primary Dysmenorrhea The recommended dose is 120 mg once daily. Minor Dental Procedures The recommended dose is 90 mg once daily. Doses greater than those recommended for each indication have either not demonstrated additional efficacy or have not been studied. Therefore: The dose for OA should not exceed 60 mg daily. The dose for RA should not exceed 90 mg daily. The dose for ankylosing spondylitis should not exceed 90 mg daily. The dose for acute gout should not exceed 120 mg daily. The dose for acute pain and primary dysmenorrhea should not exceed 120 mg daily. The dose for minor dental procedures should not exceed 90 mg daily. As the cardiovascular risks of selective COX-2 inhibitors may increase with dose and duration of exposure, the shortest duration possible and the lowest effective daily dose should be used. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically (see PRECAUTIONS). Elderly, Gender, Race No dosage adjustment in Etoricoxib is necessary for the elderly or based on gender or race. Hepatic Insufficiency In patients with mild hepatic insufficiency (Child-Pugh score 5-6), a dose of 60 mg once daily should not be exceeded. In patients with moderate hepatic insufficiency (Child-Pugh score 7- 9), the dose should be reduced; a dose of 60 mg every other day should not be exceeded, administration of 30 mg once daily can also be considered. There are no clinical or pharmacokinetic data in patients with severe hepatic insufficiency (Child-Pugh score >9). (see PRECAUTIONS). Renal Insufficiency In patients with advanced renal disease (creatinine clearance <30 mL/min), treatment with Etoricoxib is not recommended. No dosage adjustment is necessary for patients with lesser degrees of renal insufficiency (creatinine clearance ≥30 mL/min). (see PRECAUTIONS). ROUTE OF ADMINISTRATION: For oral use CONTRAINDICATIONS Etoricoxib is contraindicated in patients with: • Hypersensitivity to any component of this product. • Congestive heart failure (NYHA II-IV). • Established ischemic heart disease, peripheral arterial disease and/or cerebrovascular disease (including patients who have recently undergone coronary artery bypass graft surgery or angioplasty). • Patients with hypertension whose blood pressure has not been adequately controlled. • Patients with active peptic ulceration or gastro-intestinal (GI) bleeding • Patients with severe hepatic dysfunction (Child-Pugh score >9) • Patients with estimated creatinine clearance <30 ml/min • Patients who have developed signs of bronchospasm, acute rhinitis, nasal polyps, angioneurotic oedema or urticaria following the administration of acetylsalicylic acid or other non-steroidal anti-inflammatory drugs (NSAIDs) • Pregnancy and lactation • Children and adolescents under 16 years of age • Patients with inflammatory bowel disease WARNINGS AND PRECAUTIONS			
The use of etoricoxib, as with any medicinal product known to inhibit cyclooxygenase prostaglandin synthesis, is not recommended in women attempting to conceive. Brutaflam Tablets contains lactose. Patients with rare hereditary problems of galactose intolerance e.g., galactosaemia should not take this medicine. INTERACTIONS WITH OTHER MEDICAMENTS Warfarin: In subjects stabilized on chronic warfarin therapy, the administration of Etoricoxib 120 mg daily was associated with an approximate 13% increase in prothrombin time International Normalized Ratio (INR). Standard monitoring of INR values should be conducted when therapy with Etoricoxib is initiated or changed, particularly in the first few days, in patients receiving warfarin or similar agents. Rifampin: Co-administration of Etoricoxib with rifampin, a potent inducer of hepatic metabolism, produced a 65% decrease in etoricoxib plasma area under the curve (AUC). This interaction should be considered when Etoricoxib is co-administered with rifampin. Methotrexate: Two studies investigated the effects of Etoricoxib 60, 90 or 120 mg administered once daily for seven days in patients receiving once-weekly methotrexate doses of 7.5 to 20 mg for rheumatoid arthritis. Etoricoxib at 60 and 90 mg had no effect on methotrexate plasma concentrations (as measured by AUC) or renal clearance. In one study, Etoricoxib 120 mg had no effect on methotrexate plasma concentrations (as measured by AUC) or renal clearance. In the other study, Etoricoxib 120 mg increased methotrexate plasma concentrations by 28% (as measured by AUC) and reduced renal clearance of methotrexate by 13%. Monitoring for methotrexate-related toxicity should be considered when Etoricoxib at doses greater than 90 mg daily and methotrexate are administered concomitantly. Diuretics, Angiotensin Converting Enzyme (ACE) Inhibitors and Angiotensin II Antagonists (AIIAs): Reports suggest that NSAIDs including selective COX-2 inhibitors may diminish the antihypertensive effect of diuretics, ACE inhibitors and AIIAs. This interaction should be given consideration in patients taking Etoricoxib concomitantly with these products. In some patients with compromised renal function (e.g., elderly patients or patients who are volume-depleted, including those on diuretic therapy) who are being treated with non- steroidal anti-inflammatory drugs, including selective COX-2 inhibitors, the co-administration of ACE inhibitors or AIIAs may result in a further deterioration of renal function, including possible acute renal failure. These effects are usually reversible. Therefore, the combination should be administered with caution, especially in the elderly. Lithium: Reports suggest that non-selective NSAIDs and selective COX-2 inhibitors may increase plasma lithium levels. This interaction should be given consideration in patients taking Etoricoxib concomitantly with lithium. Aspirin: Etoricoxib can be used concomitantly with low-dose aspirin at doses for cardiovascular prophylaxis. At steady state, etoricoxib 120 mg once daily had no effect on the anti-platelet activity of low-dose aspirin (81 mg once daily). However, concomitant administration of low- dose aspirin with Etoricoxib increases the rate of GI ulceration or other complications compared to use of Etoricoxib alone. (see PRECAUTIONS). Oral Contraceptives: Etoricoxib 60mg given concomitantly with an oral contraceptive containing 35 mcg ethinyl estradiol (EE) and 0.5 to 1 mg norethindrone for 21 days increased the steady state AUC_{0-24h} of EE by 37%. Etoricoxib 120 mg given with the same oral contraceptive either concomitantly or separated by 12 hours, increased the steady state AUC_{0-24h} of EE by 50 to 60%. This increase in EE concentration should be considered when selecting an appropriate oral contraceptive for use with Etoricoxib. An increase in EE exposure can increase the incidence of adverse events associated with oral contraceptives (e.g., venous thrombo-embolic events in women at risk). Hormone Replacement Therapy: Administration of Etoricoxib 120 mg with hormone replacement therapy consisting of conjugated estrogens (0.625 mg PREMARIN®) for 28 days, increased the mean steady state AUC_{0-24h} of unconjugated estrone (41%), equilin (76%) and 17-β-estradiol (22%). The effect of the recommended chronic doses of Etoricoxib (30, 60 and 90 mg) has not been studied. The effects of Etoricoxib 120mg on the exposure (AUC_{0-24hr}) to these estrogenic components of PREMARIN® were less than half of those observed when PREMARIN® was administered alone and the dose was increased from 0.625 to 1.25 mg. The clinical significance of these increases is unknown, and higher doses of PREMARIN® were not studied in combination with Etoricoxib. These increases in estrogenic concentration should be taken into consideration when selecting post-menopausal hormone therapy for use with Etoricoxib. Other: In drug-interaction studies, Etoricoxib did not have clinically important effects on the pharmacokinetics of prednisone/prednisolone or digoxin. Antacids and ketocoazole (a potent inhibitor of CYP3A4) did not have clinically important effects on the pharmacokinetics of Etoricoxib. Cyclosporin and tacrolimus: Coadministration of cyclosporin or tacrolimus with any NSAID may increase the nephrotoxic effect of cyclosporin or tacrolimus. Renal function should be monitored when etoricoxib and either of these drugs is used in combination. Prednisone/prednisolone: Etoricoxib did not have clinically important effects on the pharmacokinetics of prednisone/prednisolone. Digoxin: Patients at high risk of digoxin toxicity should be monitored for this when etoricoxib and digoxin are administered concomitantly. Effect of etoricoxib on drugs metabolised by sulfo transferases: Etoricoxib is an inhibitor of human sulfo transferase activity, particularly SULT1E1, and has been shown to increase the serum concentrations of ethinyl estradiol. While knowledge about effects of multiple sulfo transferases is presently limited and the clinical consequences for many drugs are still being examined, it may be prudent to exercise care when administering etoricoxib concurrently with other drugs primarily metabolised by human sulfo transferases (e.g., oral salbutamol and minoxidil). Effect of etoricoxib on drugs metabolised by CYP isoenzymes: Etoricoxib is not expected to inhibit cytochromes P450 (CYP) 1A2, 2C9, 2C19, 2D6, 2E1 or 3A4. Effects of other drugs on the pharmacokinetics of etoricoxib: The main pathway of etoricoxib metabolism is dependent on CYP			

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Warnings

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAIDS

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAIDs therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Precautions

Clinical trials suggest that the selective COX-2 inhibitor class of drugs may be associated with an increased risk of thrombotic events (especially MI and stroke), relative to placebo and some NSAIDs (naproxen). As the cardiovascular risks of selective COX-2 inhibitors may increase with dose and duration of exposure, the shortest duration possible and the lowest effective daily dose should be used. The patient's need for symptomatic relief and response to therapy should be re- evaluated periodically.

Patients with significant risk factors for cardiovascular events (e.g. hypertension, hyperlipidaemia, diabetes mellitus, smoking) should only be treated with etoricoxib after careful consideration.

Selective COX-2 inhibitors are not a substitute for aspirin for cardiovascular prophylaxis because of their lack of effect on platelets. Because etoricoxib, a member of this class, does not inhibit platelet aggregation, antiplatelet therapies should not be discontinued.

There is a further increase in the risk of gastrointestinal adverse effects (gastrointestinal ulceration or other gastrointestinal complications) for etoricoxib, other selective COX-2 inhibitors and NSAIDs, when taken concomitantly with acetylsalicylic acid (even at low doses). The relative difference in gastrointestinal safety between selective COX-2 inhibitors + acetylsalicylic acid vs. NSAIDs + acetylsalicylic acid has not been adequately evaluated in long-term clinical trials.

In patients with advanced renal disease, treatment with Etoricoxib is not recommended. Clinical experience in patients with estimated creatinine clearance of <30 mL/min is very limited. If therapy with Etoricoxib must be initiated in such patients, close monitoring of the patient's renal function is advisable.

Long-term administration of NSAIDs has resulted in renal papillary necrosis and other renal injury. Renal prostaglandins may play a compensatory role in the maintenance of renal perfusion. Therefore, under conditions of compromised renal perfusion, administration of

Etoricoxib may cause a reduction in prostaglandin formation and, secondarily, in renal blood flow, and thereby impair renal function. Patients at greatest risk of this response are those with pre-existing significantly impaired renal function, uncompensated heart failure, or cirrhosis. Monitoring of renal function in such patients should be considered.

Caution should be used when initiating treatment with Etoricoxib in patients with considerable dehydration. It is advisable to rehydrate patients prior to starting therapy with Etoricoxib.

As with other drugs known to inhibit prostaglandin synthesis, fluid retention, edema and hypertension have been observed in some patients taking Etoricoxib. The possibility of fluid retention, edema or hypertension should be taken into consideration when Etoricoxib is used in patients with pre-existing edema, hypertension, or heart failure. All Nonsteroidal Antinflammatory Drugs (NSAIDs), including etoricoxib, can be associated with new onset or recurrent congestive heart failure. (see SIDE EFFECTS). Etoricoxib may be associated with more frequent and severe hypertension than some other NSAIDs and selective COX-2 inhibitors, particularly at high doses. Therefore, special attention should be paid to blood pressure monitoring during treatment with etoricoxib. If blood pressure rises significantly, alternative treatment should be considered.

Physicians should be aware that individual patients may develop upper gastrointestinal (GI) ulcers/ulcer complications irrespective of treatment. Although the risk of GI toxicity is not eliminated with Etoricoxib, the results of the MEDAL Program demonstrate that in patients treated with Etoricoxib, the risk of GI toxicity with Etoricoxib 60 mg or 90 mg once daily is significantly less than that with diclofenac 150 mg daily. In clinical studies with ibuprofen and naproxen, the risk of endoscopically detected upper GI ulcers was lower in patients treated with Etoricoxib 120 mg once daily than in patients treated with the non-selective NSAIDs. While the risk of endoscopically detected ulcers was low in patients treated with Etoricoxib 120 mg it was higher than in patients treated with placebo. Upper GI ulcers/ulcer complications have occurred in patients treated with Etoricoxib. These events can occur at any time during use and without warning symptoms. Independent of treatment, patients with a prior history of GI perforation, ulcers and bleeding (PUB) and patients greater than 65 years of age are known to be at a higher risk for a PUB.

Elevations of alanine aminotransferase (ALT) and/or aspartate aminotransferase (AST) (approximately three or more times the upper limit of normal) have been reported in approximately 1% of patients in clinical trials treated for up to one year with Etoricoxib 30, 60 and 90 mg daily. In active comparator portions of clinical trials, the incidence of elevated AST and/or ALT in patients treated with Etoricoxib 60 and 90 mg daily was similar to that of patients treated with naproxen 1000 mg daily, but notably less than the incidence in the diclofenac 150 mg daily group. These elevations resolved in patients treated with Etoricoxib, with approximately half resolving while patients remained on therapy. In controlled clinical trials of Etoricoxib 30 mg daily versus ibuprofen 2400 mg daily or celecoxib 200 mg daily, the incidence of elevations of ALT or AST was similar.

A patient with symptoms and/or signs suggesting liver dysfunction, or in whom an abnormal liver function test has occurred, should be evaluated for persistently abnormal liver function tests. If persistently abnormal liver function tests (three times the upper limit of normal) are detected, Etoricoxib should be discontinued.

Etoricoxib should be used with caution in patients who have previously experienced acute asthmatic attacks, urticaria, or rhinitis, which were precipitated by salicylates or non-selective cyclooxygenase inhibitors. Since the pathophysiology of these reactions is unknown, physicians should weigh the potential benefits of prescribing Etoricoxib versus the potential risks.

When using etoricoxib in the elderly and in patients with renal, hepatic, or cardiac dysfunction, medically appropriate supervision should be maintained. If these patients deteriorate during treatment, appropriate measures should be taken, including discontinuation of therapy.

Serious skin reactions, some of them fatal, including exfoliative dermatitis, Stevens-Johnson syndrome, and toxic epidermal necrolysis, have been reported very rarely in association with the use of NSAIDs and some selective COX-2 inhibitors during post-marketing surveillance (see SIDE EFFECTS). These serious events may occur without warning. Patients appear to be at highest risk for these reactions early in the course of therapy; the onset of the reaction occurring in the majority of cases within the first month of treatment. Serious hypersensitivity reactions (such as anaphylaxis and angioedema) have been reported in patients receiving etoricoxib (see SIDE EFFECTS). Some selective COX-2 inhibitors have been associated with an increased risk of skin reactions in patients with a history of any drug allergy. Etoricoxib should be discontinued at the first appearance of skin rash, mucosal lesions, or any other sign of hypersensitivity.

Etoricoxib may mask fever, which is a sign of infection. The physician should be aware of this when using Etoricoxib in patients being treated for infection.

Cardiac disorders: congestive heart failure, palpitations, angina, arrhythmia.

Vascular disorders: hypertensive crisis.

Respiratory, thoracic and mediastinal disorders: bronchospasm.

Gastrointestinal disorders: abdominal pain, oral ulcers, peptic ulcers including perforation and bleeding (mainly in elderly patients), vomiting, diarrhea.

Hepatobiliary disorders: hepatitis, jaundice, hepatic failure.

Skin and subcutaneous tissue disorders: angioedema, pruritus, erythema, rash, Stevens- Johnson syndrome, toxic epidermal necrolysis, urticaria, fixed drug eruption.

Renal and urinary disorders: renal insufficiency, including renal failure (see PRECAUTIONS).

SYMPTOMS AND TREATMENT OF OVERDOSE

In clinical studies, administration of Etoricoxib at single doses up to 500 mg and multiple doses up to 150 mg/day for 21 days did not result in significant toxicity. There have been reports of acute overdose with etoricoxib, although adverse experiences were not reported in the majority of cases. The most frequently observed adverse experience were consistent with the safety profile for etoricoxib (e.g. gastrointestinal events, renovascular events).

In the event of overdose, it is reasonable to employ the usual supportive measures, e.g., remove unabsorbed material from the gastrointestinal tract, employ clinical monitoring, and institute supportive therapy, if required.

Etoricoxib is not dialyzable by hemodialysis; it is not known whether etoricoxib is dialyzable by peritoneal dialysis.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Patients who experience dizziness, vertigo or somnolence while taking etoricoxib should refrain from driving or operating machinery.

STORAGE CONDITIONS

Store below 30°C. Protect from light & moisture.

Keep medicine out of reach of children.

SHELF LIFE

36 months from the date of manufacture.

DOSAGE FORMS AND PACKAGING AVAILABLE

10 Tablets packed in Alu/Alu blister. 3 such blisters are packed in a carton along with package insert.

PRODUCT REGISTRATION HOLDER

Pahang Pharmacy Sdn. Bhd.

Lot 5979, Jalan Teratai,

5 ½ Miles off Jalan Meru, 41050

Klang, Selangor, Malaysia

NAME AND ADDRESS OF MANUFACTURER



Manufactured by :

MANKIND PHARMA LTD.

Unit-II, Village-Kishanpura, P.O. Jamniwala,

Tehsil-Paonta Sahib, District Sirmour-173 025,

Himachal Pradesh (INDIA)

Regd. Office :

208, Okhla Industrial Estate, Phase-3,

New Delhi-110 020 (INDIA)

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